

Provider Net Cost Model Concept Brief and Launch Access Reality Addendum for Adrabetadex

Background and core assumption

This package is written in the context of Navisync ¹ supporting Beren Therapeutics, PBC ² in advance of an expected August 2026 PDUFA decision for Adrabetadex, with the immediate objective of level-setting (for a non-reimbursement audience) why “Expanded Access Program (EAP) experience” is not an analytically safe proxy for post-launch provider economics. ³

Under expanded access, the operational gatekeeper is *manufacturer willingness to supply the product*, not payer coverage. U.S. Food and Drug Administration ⁴ describes that, before a physician can submit a single-patient expanded access request, the physician “must first ensure that the manufacturer is willing to provide the investigational drug.” ⁵ Under FDA’s expanded access framework, FDA must determine (among other criteria) that the patient has a serious or immediately life-threatening disease, that there is no comparable or satisfactory alternative therapy, that potential benefit justifies potential risk, and that expanded access will not interfere with clinical investigations that could support marketing approval. ⁶

Expanded access also permits—but tightly regulates—charging for the investigational drug. The charging standard is fundamentally different from a commercial “buy-and-bill” world: sponsors may recover only *direct costs* of making the investigational drug available (and certain additional administrative costs only for some expanded access categories), not a market price, and FDA authorization is required to charge under these provisions. ⁷ This is one of the main reasons EAP-era “reimbursement experience” can appear artificially unfriktioned (or at least non-generalizable): the drug supply and pricing mechanics are not payer-adjudicated the way they will be after approval. ⁸

Post-launch, the access paradigm is expected to shift from manufacturer-granted investigational supply to a payer-mediated, documentation- and policy-driven reimbursement framework. Even when clinical value is compelling, payers commonly apply: (1) coverage criteria and prior authorization requirements, (2) site-of-care steering, and (3) distribution channel controls (e.g., payer-directed specialty pharmacy “white bagging” / sourcing protocols). Evidence of these controls is visible in publicly available commercial payer policies, including explicit statements that payment can be denied if the provider does not follow a required sourcing protocol. ⁹

For planning purposes, Beren’s stated list price and regimen create an unusually large provider “float” and denial exposure if the product is treated as a traditional buy-and-bill therapy. The internal draft discussion references a WAC of \$39,000 per 900 mg vial administered every two weeks (i.e., high recurring acquisition exposure). ¹⁰

Document A: provider net cost model concept brief

A provider-based net cost model for Adrabetadex is a structured, scenario-driven calculator that estimates —by payer type and site of care—whether providers (and, specifically, centers of excellence) can reliably recover drug acquisition costs and administration costs *on a cash-flow and denial-adjusted basis*, rather than on an idealized “allowed amount” basis. The model’s strategic value is that it converts a clinical story into an operational economics story that matches how COEs, revenue cycle teams, and pharmacy leaders decide whether they can scale treatment. ¹⁰

The simplest reason this tool matters is that “buy-and-bill” is not only a pricing question; it is a workflow and risk-transfer construct. Under buy-and-bill, providers acquire inventory (often through a wholesaler), store/ manage it, administer it, and then bill payers—meaning providers carry *coding risk, coverage risk, and working-capital risk*. ICER’s policy analysis describes buy-and-bill as providers purchasing drugs from a wholesaler and then being responsible for storing/administering them to patients. ¹¹ As payer strategies evolve, the “buy-and-bill” channel is increasingly challenged by payer efforts to reduce provider markup and steer utilization through alternative distribution models (white bagging) and site-of-service policies. ¹²

A provider net cost model concept brief (for MLR and provider stakeholders) should be explicit about what the model *is* and *is not*:

- It is a planning tool to quantify provider economics sensitivity to payer/payment mechanics (coverage criteria, coding, site-of-care, timing, and distribution), not a guarantee of any payer outcome. ¹³
- It is designed to surface where EAP-era experience diverges from post-approval reality, especially where drug cost is currently absorbed by the manufacturer rather than financed and recovered by providers through billed claims. ⁸
- It is a modular framework that supports different payer archetypes (commercial, Medicaid FFS, Medicaid managed care, and 340B hospitals), because reimbursement and acquisition economics differ materially across those segments. ¹⁴

The concept brief should also define, in plain language, several terms that will recur in the model and will likely be unfamiliar to non-access stakeholders:

- **WAC** (Wholesale Acquisition Cost): CMS describes WAC as the manufacturer’s list price to wholesalers/direct purchasers, excluding prompt-pay discounts, rebates, or other reductions. CMS also notes WAC is typically higher than ASP (magnitude varies). ¹⁵
- **ASP-based reimbursement**: CMS states that Medicare pays most separately payable Part B drugs at ASP + 6%. ¹⁶
- **New drug WAC-based reimbursement period**: CMS notes that during the initial sales period when ASP is not yet available, the payment limit is 103% of WAC. ¹⁷
- **White bagging / sourcing protocol**: UnitedHealthcare defines “white bagging” as purchasing through a specialty pharmacy and shipping to the provider for administration; it also states that if the provider does not obtain the specialty drug through an indicated specialty pharmacy, UnitedHealthcare will deny payment for the medication. ¹⁸
- **340B ceiling price**: HRSA explains that the statutory ceiling price is AMP minus URA. ¹⁹

Model methodology and data requirements

The model should be built as a “module stack” that starts with drug acquisition and ends with *net cost recovery*, with the ability to toggle assumptions for payer type, channel, and site of care. The goal is reproducible calculations that an MLR audience can audit conceptually, and that providers can recognize as aligned to real billing flows. ²⁰

Core calculation concept (single administration event):

Net Cost Recovery (per administration) = *(Expected drug revenue + expected admin/procedure revenue) – (drug acquisition + expected non-reimbursed drug loss + admin/procedure delivery cost + expected denial/rework cost + cost of capital on receivables)*. ²¹

To keep this MLR-safe and provider-relevant, the concept brief should show how each term maps to public reimbursement mechanics:

Drug revenue benchmarks (for anchoring, not as a prediction for pediatrics):

Medicare Part B is a widely used benchmark for clinician-administered drug reimbursement mechanics. CMS and MedPAC both describe the general “ASP + 6%” framework for most separately payable Part B drugs. ¹⁶ For newly marketed products (when ASP is not yet available), CMS describes the “103% of WAC” payment limit during the initial sales period. ¹⁷

Hospital outpatient payment and early coding risk:

For hospital outpatient departments under OPPS, whether a drug is paid separately, packaged, or temporarily eligible for pass-through is not a “given”—it is policy- and coding-dependent. MedPAC explains that OPPS typically packages ancillary items into a payment for the primary service, and separately pays some higher-cost drugs through pass-through and separately payable non-pass-through drug policies. ²² CMS similarly describes transitional pass-through payments as temporary additional payments for certain new drugs/biologicals (generally 2–3 years) to facilitate patient access while CMS gathers cost data, and provides criteria for when drug costs are “not insignificant.” ²³

For drugs without a product-specific HCPCS code, billing instructions and documentation rules can directly affect payment. CMS contractors have published explicit guidance distinguishing when to use C9399 (for “new” FDA-approved drugs without a specific code) versus NOC codes, and emphasize that missing documentation can prevent payment—citing Title XVIII §1833(e). ²⁴ For NOC/unclassified drug codes, CMS contractor guidance also specifies that claims must include information such as drug name, NDC number, and total dosage, and that narrative documentation may be required to confirm label-appropriate use. ²⁵

340B acquisition economics module:

For centers of excellence that are 340B covered entities, drug acquisition economics can diverge dramatically from non-340B providers. HRSA describes the ceiling price as AMP minus URA. ¹⁹ For new drugs, HRSA’s manufacturer help guidance states that manufacturers must estimate a new drug’s ceiling price as WAC minus the appropriate rebate percentage until AMP is available (no later than the fourth quarter the drug is available for sale). ²⁶ This provides a concrete, publicly defensible basis for “new drug” 340B scenario assumptions in an early launch model, even before real AMP/URA values are known. ²⁶

Because 340B and Medicaid rebates interact, the model must also account for duplicate discount risk and operational carve-in/carve-out mechanics. HRSA explains that federal law prohibits “duplicate discounts” (a 340B discount and a Medicaid drug rebate for the same drug) and requires covered entities to have mechanisms to prevent this; HRSA also describes how carve-in/carve-out decisions are reflected in the Medicaid Exclusion File. ²⁷

Medicaid module (pediatrics-specific operational reality):

Even where EPSDT creates a strong coverage entitlement framework for medically necessary services for children, that does not remove billing mechanics, NDC requirements, or prior authorization workflows. Medicaid.gov explains that states are required to furnish medically necessary 1905(a) services needed to “correct and ameliorate” conditions under EPSDT (including additional coverable services even if not covered in the state plan). ²⁸ Separately, Medicaid.gov explains that, under the Deficit Reduction Act (DRA), states must collect and submit utilization data (and, for defined categories, NDCs) for physician-administered drugs as a condition of Medicaid payment availability and federal matching funds, explicitly tying payment availability to NDC/utilization data capture. ²⁹

Intrathecal administration and “procedure cost” considerations:

A provider net cost model for an intrathecal therapy must treat administration as more than a generic infusion fee. Public payer policies for other intrathecally administered ultra-rare therapies illustrate the operational reality: imaging guidance, anesthesia/sedation, and specialized procedural capability can be part of routine administration pathways. For example, Aetna’s clinical policy bulletin for nusinersen includes CPT codes and references related to lumbar puncture, subarachnoid injection procedures with and without imaging guidance, and fluoroscopic guidance—reflecting that intrathecal administration frequently relies on specialized procedure infrastructure. ³⁰

Deliverable format and governance

The recommended deliverable should be a provider-facing calculator (spreadsheet-based) accompanied by a short, citation-supported concept brief for internal governance stakeholders (MLR, compliance, medical). This pairing is important because reimbursement discussions are often treated as “high sensitivity” internally, and the brief can document the model’s intended use, key assumptions, and boundaries. ³¹

A pragmatic format is:

- **Excel (or equivalent) calculator** with locked formula cells and clearly labeled input tabs for: product assumptions (dose/vial utilization, WAC), payer scenario assumptions (allowed % of benchmark, denial rates, days-to-payment), site-of-care (HOPD vs physician office), 340B status (non-340B vs 340B new drug estimate), and Medicaid requirements (NDC capture sensitivity). ³²
- **Outputs dashboard** that presents (by scenario) net margin per administration, break-even points, and “cash exposure” metrics (e.g., peak receivable exposure based on payment timing), along with a concise narrative explaining what drives variation. ³³
- **A short memo-style concept brief** that cites: (1) expanded access mechanics and charging limitations (to highlight the pre/post launch paradigm shift), (2) Medicare and OPPS payment benchmarks (to ground how buy-and-bill is typically paid and documented), (3) 340B ceiling price and new drug estimation rules, and (4) Medicaid PAD and EPSDT requirements. ³⁴

Governance language should be non-promotional and operational: CMS and contractor publications themselves emphasize that correct documentation and coding is required for claim processing and that missing documentation can bar payment; incorporating those exact mechanics into the model is therefore defensible as “billing reality,” not messaging. ¹³

Document B: U.S. reimbursement landscape and launch friction addendum

This addendum is an educational explainer—designed for a non-reimbursement MLR audience—covering the most likely launch-era opportunities and constraints for an expensive, clinician-administered therapy in an ultra-rare pediatric indication, with emphasis on (a) buy-and-bill realities in a vertically integrated payer environment, (b) Medicaid + EPSDT operationalization, and (c) 340B economics.

A central theme is that “payers will cover it because outcomes are strong” is not the same as “providers can operationalize it without financial risk.” The commercial market has developed multiple levers that can preserve coverage in principle while still constraining channel, site, and operational burden in practice. ICER describes payer strategies increasingly aimed at reducing provider markup by bypassing buy-and-bill through payer-affiliated specialty pharmacies (“white bagging”) and by using site-of-service policies to require administration in lower-cost settings. ³⁵

Commercial market friction examples that are visible today

Even in rare neurologic diseases, major payers publish explicit medical necessity criteria requiring genetic confirmation, specialist involvement, baseline assessments, and proof of ongoing benefit for reauthorization. For example, UnitedHealthcare’s medical necessity criteria for arimoclomol (Miplyffa) for Niemann-Pick disease type C require genetic confirmation (NPC1/NPC2 mutation analysis), specialist prescribing/consultation, and documentation of positive clinical response for reauthorization. ³⁶ Similar criteria appear for levacetyleucine (Aqneursa), again requiring genetic confirmation and documentation of positive response for continued authorization. ³⁷

Comparable patterns are visible in other ultra-rare, clinician-administered neurologic therapies. Aetna’s nusinersen policy, for example, requires genetic documentation and baseline assessments prior to therapy initiation, and the policy text reflects the administrative structure of “baseline + continuation” decisions. ³⁸ For Brineura (cerliponase alfa), Aetna’s policy requires administration by (or under direction of) a physician knowledgeable in intraventricular administration and applies continuation criteria tied to functional outcomes. ³⁹

These examples matter for Adrabetadex not because the criteria will match 1:1, but because they demonstrate the *shape* of real-world payer behavior: high-need populations still face structured utilization management and reauthorization frameworks, and providers must staff to that operational burden. ⁴⁰

Vertically integrated controls affecting buy-and-bill

Public sources also document the market structure that enables “channel enforcement.” The U.S. Government Accountability Office notes that the largest PBMs are fully vertically integrated with large health insurers and wholly owned mail order and specialty pharmacies, and also describes PBM roles in

utilization management such as prior authorization. ⁴¹ The Federal Trade Commission has also published staff reports focused on vertically integrated PBMs, reflecting ongoing regulatory attention to PBM incentives and behaviors. ⁴²

At the policy level, this vertical integration shows up as explicit payer requirements that can re-route clinician-administered drugs away from traditional buy-and-bill. UnitedHealthcare's medication sourcing protocol FAQ states that outpatient providers may only bill for administering the medication (not the drug), that the specialty pharmacy bills UnitedHealthcare directly, and that UnitedHealthcare will deny payment if the drug is not obtained through an indicated specialty pharmacy. ⁴³ This is a direct, publicly documented example of how "provider economics" can be transformed even when a drug is covered under the medical benefit: the provider may be structurally prevented from earning any drug margin and may still bear operational responsibilities around scheduling and administration. ⁴⁴

Similarly, UnitedHealthcare's site-of-care policy for provider-administered drugs describes criteria under which outpatient hospital facility-based infusion is allowed and emphasizes that alternative sites of care (physician office, ambulatory infusion suite, home infusion) are "well accepted," with documentation required to justify hospital outpatient administration and time-limited approvals for hospital outpatient infusion. ⁴⁵ While Adrabetadex is intrathecal (not simple infusion), the policy illustrates a mainstream payer posture: site-of-care is explicitly managed, and "hospital-based" delivery often requires additional justification. ⁴⁶

Medicaid and pediatric reality

EPSDT is an important tailwind for pediatric access, but it is not an operational shortcut. Medicaid.gov states that states must furnish medically necessary services needed to "correct and ameliorate" conditions for beneficiaries under 21, including additional coverable services regardless of whether the service is covered in the state plan. ⁴⁷ In parallel, Medicaid physician-administered drug payment has specific rebate-driven data requirements: the Medicaid.gov PAD page ties Medicaid payment availability (and FFP) to states collecting and submitting utilization data and, for defined categories, NDCs. ²⁹

For an intrathecally administered, high-WAC product, these points translate into practical launch frictions that a provider net cost model should explicitly quantify as sensitivities: NDC capture/claim formatting error rates, prior authorization lead times, and denial/rework costs—because these are the mechanics by which "coverage" becomes (or fails to become) "cash." ⁴⁸

340B opportunities and compliance constraints

The 340B landscape creates a real economic opportunity for eligible COEs, while simultaneously adding program integrity requirements that affect Medicaid billing and audit exposure. HRSA explains that duplicate discounts are prohibited and that covered entities must maintain mechanisms to prevent them, including accurate Medicaid carve-in/carve-out reporting reflected in the Medicaid Exclusion File. ²⁷ HRSA also provides a concrete rule-based approach for estimating 340B ceiling prices for new drugs (WAC minus appropriate rebate percentage until AMP is available), which can be used to structure an early launch "340B acquisition" scenario in the model. ²⁶

The implication for Adrabetadex is that 340B is not just a "discount"; it is a scenario that can change provider margin markedly while introducing additional operational requirements (carve-in/out, claim-level

practices, audit readiness). Those operational requirements are sufficiently material that, for COEs, the net cost model should present 340B and non-340B scenarios separately rather than averaging them together.

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Blind spot check

A plausible internal counter-position is: “Because the disease is catastrophic and the effect size is strong, payer friction will be minimal; therefore a detailed provider net cost recovery model is unnecessary.” The public record of payer behavior in ultra-rare neurologic diseases does not support that simplification. UnitedHealthcare’s NPC criteria (for Miplyffa/Aqneursa) require genetic confirmation, specialist involvement, and documentation of benefit for reauthorization—demonstrating that even in rare, serious disease, payers operationalize access through structured criteria. ⁵⁰ Aetna’s policies for intrathecal/intraventricular rare disease therapies similarly apply baseline assessment requirements and continuation criteria tied to clinical status. ⁵¹

A second counter-position is: “COEs already treat under EAP; therefore launch will look similar.” FDA’s own expanded access materials make clear that expanded access begins with the manufacturer agreeing to provide the investigational drug; it is not a payer-driven acquisition channel. ³ Once the product transitions into commercial distribution, publicly documented payer controls—such as mandatory specialty pharmacy sourcing with explicit denial of payment for noncompliance—can materially change provider economics versus the EAP period. ⁵²

A third counter-position is: “Even if buy-and-bill is the pathway, reimbursement will ‘work itself out’ quickly.” CMS contractor guidance on unclassified/NOC drug billing illustrates why early launch is often a *coding + documentation* problem before it is a *coverage* problem: missing or insufficient documentation can bar payment (citing §1833(e)), and NOC/unclassified claims have explicit documentation requirements (drug name, NDC, dosage; and, in some contexts, narrative proof of labeled-criteria). ⁵³ This supports the core premise of the provider net cost recovery model: provider economics at launch are meaningfully shaped by operational frictions (coding, documentation, denials, cash timing), not only by clinical value or a headline reimbursement benchmark. ⁵⁴

¹ ²⁶ New Drug Price Estimation

<https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/New%20Drug%20Price%20Estimation.htm>

² ⁹ ¹⁸ ⁴³ ⁴⁴ ⁵² Medication sourcing protocol – Requirements to use a participating specialty pharmacy for certain medications - UnitedHealthcare commercial plans

<https://www.uhcprovider.com/content/dam/provider/docs/public/resources/pharmacy/medication-expansion-sourcing-faq.pdf>

³ ⁵ ⁸ ³⁴ For Physicians: How to Request Single Patient Expanded Access (“Compassionate Use”) | FDA

<https://www.fda.gov/drugs/investigational-new-drug-ind-application/physicians-how-request-single-patient-expanded-access-compassionate-use>

⁴ ¹⁶ Medicare Part B Drug Average Sales Price

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